

Schedule of assumed facts

WC/2024/227 · Shepherd · accompanying the letter of instruction · [DATE]

This schedule identifies, by source and paragraph, the facts referred to in Part B of the letter of instruction. **The documents themselves accompany it, complete and unmarked.** Where a passage is quoted, the quotation is exact. **Section B records the matters the Workers' Compensation Regulator does *not* accept, so that the position is complete.**

A · ACCEPTED BY THE WORKERS' COMPENSATION REGULATOR — RESPONSE SIGNED BY MS RENEE MATHESON, SENIOR APPEALS OFFICER, 18 FEBRUARY 2026

Item numbers are those of the list to which that document responds; the cross-references have been checked against it. These are facts the Regulator accepts.

Item	Fact accepted
1	On 17–18 March 2024 the appellant was rostered to finish at 23:00 and commence his next shift at 06:00, a break of only 7 hours.
3	The employer's fatigue risk management policy and the relevant Award require a minimum break of 10 hours between shifts, or 8 hours by written agreement.
5	Ms Tammy Reese stated in an email of 7 August 2023 : <i>"I do realise in this instance there was a rostering error that was accidentally made by Chloe with regards to night shifts."</i>
8	Maintaining accurate contact details for medical staff is a critical function of the Switchboard to ensure effective clinical handover and patient safety. It adds that <i>"there was a procedure in place for this to occur."</i>
20	On 24 December 2024 the Ethical Standards Unit determined that the appellant's complaint regarding Ms Taylor and Ms Reese constituted a Public Interest Disclosure.
33	The clinical records contain a consultation record dated 16 November 2023 authored by Dr Priyal De Silva Nanayakkara. (<i>Accepted as being in the record; its accuracy is not accepted — see B.</i>)
38	The report of Dr Ravikumar Krishnaiah states that the appellant was suffering from Major Depressive Disorder, diagnosed 13 February 2025. (<i>Accepted as what the report says; its accuracy is not accepted.</i>)
40	On 3 May 2024 Payroll Officer Elaine Grant emailed the line manager instructing her to <i>"submit an AVAC to correct these shifts"</i> for the appellant.
41	The AVAC was submitted on 28 May 2024. It adds that on 21 May the line manager had said she was waiting on payroll confirmation.
46	On 21 May 2024 at 12:33pm the line manager emailed the appellant: <i>"I am waiting payroll confirmation... as soon as I do get that confirmation, I will submit an AVAC."</i>

A2 · FROM A FURTHER DOCUMENT OF THE WORKERS' COMPENSATION REGULATOR, 13 MAY 2026

These are its own positive statements in that document.

Item	What it records
14(e) 14(f)	On the special pandemic leave application (leave of 20–27 February 2024), which was declined for want of an attached statutory declaration: <i>"says that a review indicates that in fact, the attachments were present on the appellant's submission"</i> · <i>"says this was a matter of human error by Ms Taylor on the background of high work demands surrounding the management of staff with COVID-19".</i>
22(a)	On the rostering of 17–18 March 2024: <i>"says the shift was separated by only a 7-hour break (rather than an 8-hour break) says that this was a result of human error and not intentional or repeated".</i>
22(c)	<i>"says the appellant took leave on 19 March 2024", and says this was paid leave.</i>

B · NOT ACCEPTED — AND WHAT THE REGULATOR SAYS INSTEAD

Recorded so that no fact in section A is read as broader than it is.

Item	Its position
4	Not accepted that there was no written agreement: <i>"in June 2020, the Appellant signed an agreement allowing an 8 hour break between shifts."</i> On that case the applicable minimum was 8 hours, and the break given was 7.

Item	Its position
34	Accepts the 16 November 2023 entry (" <i>No psychological illness such as depression/ psychosis</i> ") is in the record, but does not accept its accuracy.
35	Not accepted. It asserts a past medical history of anxiety and ADHD from 26 October 2022.
47	Not accepted that the line manager had simply held the instruction: it says " <i>Ms Taylor needed confirmation from payroll and then she needed to be satisfied that the AVAC was accurate.</i> " (The two emails at items 40 and 46 are accepted.)

C · FROM THE LETTER OF THE CHIEF EXECUTIVE, METRO SOUTH HEALTH, 5 JUNE 2026

Item numbers are those of the letter. Read from source.

Item	What the letter records
1–2	Pre-upgrade SPOK records were not retained. "However, a spreadsheet of recorded MET calls is available for the period 17-18 March 2024."
3(a)	<i>"All employee complaints relating to Logan Hospital Switchboard operational errors are made directly to the Line Manager of Switch Board and managed solely via email or verbally with the complainant."</i>
3(c)	<i>"there have been no 'consequential' changes to operating procedures over the period requested."</i>
4	The requested fatigue risk assessment records do not exist. <i>"Mandatory Fatigue Risk Management System training only applies to health practitioners and clinical assistants. The Logan Hospital Switchboard staff are non-clinical staff..."</i>
5	<i>"The implementation of fatigue risk management assessment at Switchboard Logan Hospital occurred after 30 June 2024 in connection with an organisational change related to the reporting lines for Switchboard."</i>
7	The requested fatigue risk management register entries do not exist.
15	Enclosed: leave takings report for the Applicant, 19 March 2024.

D · FROM THE ROLE DESCRIPTION — ADMINISTRATION OFFICER, SWITCHBOARD SERVICES (AO3)

Quoted from the employer's own document. Read from source.

Under	The position requires
Purpose	<i>"The occupant of this position is required to work continuous shift work over the full 24-hour period, 7 days a week."</i>
Key responsibilities	<i>"Participate in the Emergency Response process by receiving emergency response notifications and distributing them to the appropriate response groups, dependent on the category of emergency, as per emergency code procedures, strictly adhering to protocols and timeframes."</i>
	<i>"Maintain call queues to minimum at all times." · "The ability to multitask and operate under pressure, particularly where high volume call traffic is concerned."</i>
	<i>"Maintain discretion and exercise judgement where necessary to resolve problems within the scope of your role; in situations where precedence have not been set and procedures not defined."</i>
	<i>"Ability to work effectively as an individual with limited supervision..."</i>
	<i>"Collate information and maintain Omnivista database and SharePoint to ensure information held within Switchboard Services is accurate and appropriate."</i>
Mandatory requirements	<i>"The position is a continuous shift working role. You must be able to work a roster which covers multiple shifts over a 24/7 period."</i>

E · FROM THE EMPLOYEE CAPABILITY CHECKLIST, 3 JULY 2026

Read from source. Quotations exact.

Question	What the checklist records
1	<i>“Major Depressive Disorder with anxious distress (DSM-5 296.23), a work-related psychological injury with onset 18 June 2024... Residual symptoms most relevant to work capacity are reduced concentration and working memory under sustained high-demand load, fatigue sensitivity, and symptom exacerbation on exposure to the identified workplace stressors (complaint handling; being held accountable and blamed for the failures of others; unpredictable rostering).”</i>
2	<i>“Episodic, with symptom exacerbation linked to stressor exposure. The restrictions below are expected to remain necessary for at least the next 3–6 months, subject to treatment response and 8-weekly review.”</i>
3	<i>Six 8-hour shifts per fortnight (of ten in a full-time fortnight) — no more than two weekday day/afternoon, two weekend day, two night. ■ “This reduced pattern of approximately six shifts per fortnight is the pattern Mr Shepherd has in fact worked and tolerated over the past twelve months without deterioration... Usual switchboard operational duties remain suitable; complaint-handling duties are excluded as below.”</i>
4	<i>“(a) No complaint-handling duties — complaints received at the Switchboard to be logged and redirected, not actioned or resolved by Mr Shepherd; (b) predictable rostering published in advance, with a minimum 10-hour break between shifts consistent with the Award and fatigue risk management standards; (c) return-to-work coordination through the Injury Management team; (d) access to EAP; (e) review in approximately 8 weeks.”</i>
Meds	<i>Fluoxetine 60mg mane · lisdexamfetamine 30mg mane for cognitive symptoms and focus · quetiapine 25mg nocte ceased. “maintaining a regular sleep routine remains central to symptom management following cessation of the night-time medication.”</i>